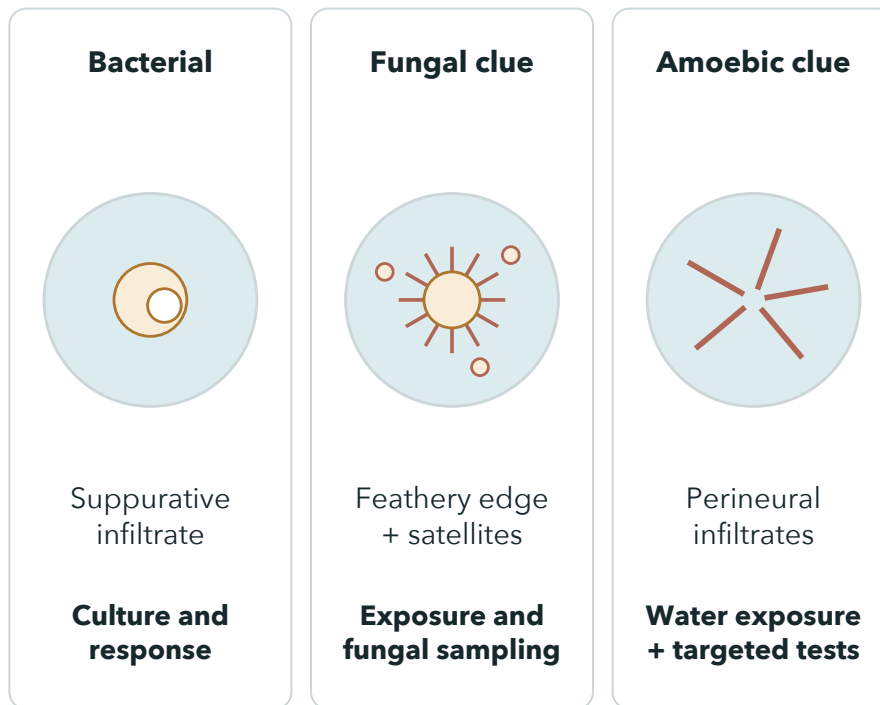


Keratitis that fails to improve

Reconsider the organism, treatment delivery and tissue integrity.

Pattern changes the sampling

A | Three clues to revisit



Original corneal surface schematics. Morphology guides suspicion; it cannot replace microbiology or specialist examination.

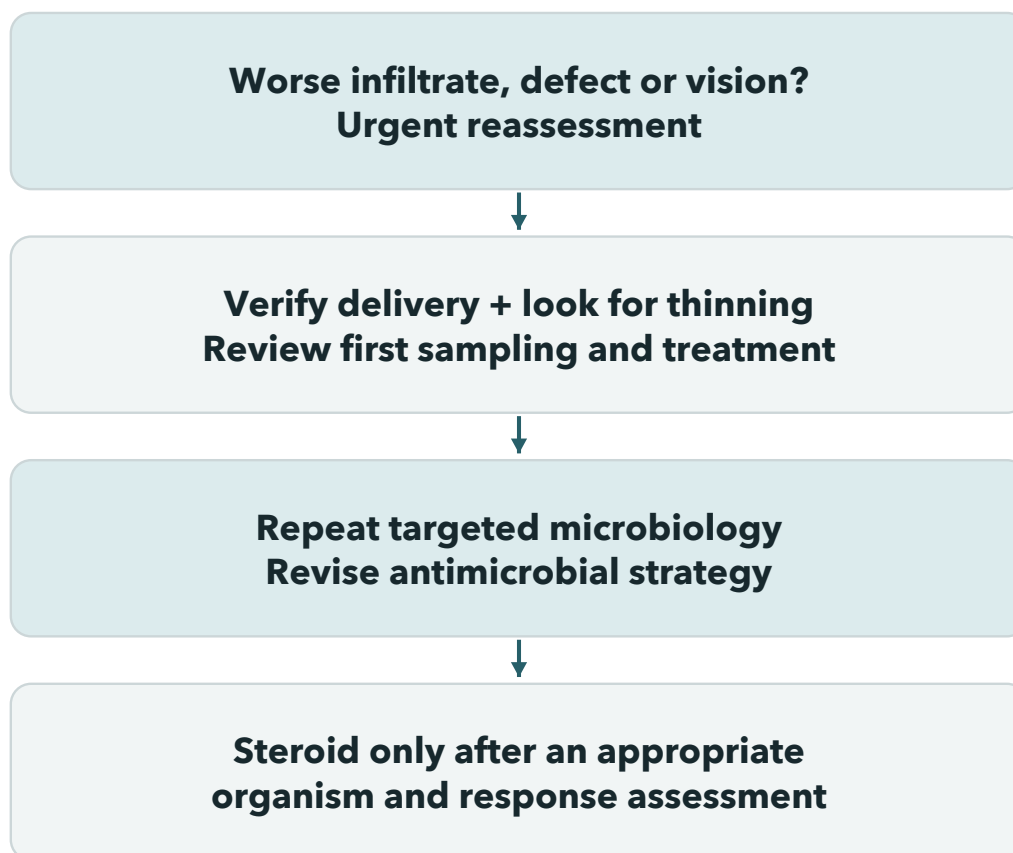
Interpret the course

Clue	Next inference
Bacterial treatment works	Less pain, smaller defect and infiltrate, improving chamber reaction support response.
Vegetative trauma; feathery edge	Consider fungal infection; request fungal microscopy and culture.
Lenses plus water; severe pain	Consider Acanthamoeba; use targeted PCR or culture and confocal imaging if available.
Negative first culture	Prior treatment or insufficient sampling can reduce yield; it does not establish a sterile ulcer.

References AAO EyeWiki | Bacterial keratitis | AAO EyeWiki | Fungal keratitis | AAO EyeWiki | Acanthamoeba keratitis

Failure needs a fresh assessment

B | Reassess before adding steroid



Original decision pathway: review the patient, prescription, organism and tissue response together.

The high-yield decision

An enlarging ulcer needs urgent cornea reassessment, repeat targeted sampling when indicated and revised antimicrobial treatment. Verify concentration, dosing and actual delivery. Assess thinning or leakage; threatened integrity may need protective or surgical measures. Adjunctive steroid is a selected decision after effective bacterial therapy and response, with antimicrobials continued and close review. Avoid blind steroid escalation in fungal, *Acanthamoeba* or *Nocardia* infection. Do not infer safety from reduced pain alone. In a contact-lens wearer whose pain persists despite observed antibacterial doses, reconsider *Acanthamoeba* testing rather than simply increasing the same bottle. After vegetative trauma, an enlarging feathery infiltrate instead directs fungal investigation. Both pathways require urgent review of tissue integrity.